

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: MEDICINE

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

You are a general practitioner. Ms Sarah Day, 20 years old, has been attending your practice since her early childhood.

Patient: Ms Sarah Day, D.O.B. 29.07.1997

Past medical history: Jul 2001: varicella
Apr 2002: measles
Jan 2004: fractured ulna
Jun 2006: URTI
Sep 2008: plantar warts
Dec 2011: dysmenorrhoea
Apr 2013: teenage acne
Aug 2014: pre-menstrual syndrome
Jun 2015: exam-related anxiety
Nov 2016: oro-facial herpes simplex virus

Social background: University student – not happy with course of study
Part-time job – McDonald's
Irregular hours, 5-8 hrs sleep/night
Smokes, drinks moderately
Lives at home with parents, boyfriend for 7 months

Medications: Oral contraceptive pill (prescribed April 2013) cyproterone acetate/ethinylestradiol – mane

06.12.17 Pt presented with mother complaining of unilateral headache – occipital, temporal extending to vertex, dizziness/loss of balance, with nausea and anxiety. Visual disturbances. Better when lying down. Symptoms not affected by red wine, alcohol, chocolate, cheese, sunlight. Not related to menstrual cycle or stress.

Pt reports workload stress, pressure of assignments & exams.

Episodes: x2 in past 2 weeks.

Pain: Steady, not throbbing.

Onset: Rapid
Aura
Symptoms last 1-2 days (severe for several hours).
No family history of headaches/migraines.

Examination: No abnormal neurological signs.

Treatment:

04.01.18

Education: rest/relaxation/meditation, adequate sleep, regular eating.
During attack: sleep, dark room, ice.
Metoclopramide 10 mg – orally → 20 mins later paracetamol 1 g – 4 hrly (max 4 g/24 hrs).

Attacks more frequent. Prescribed medications ineffective.
Pain felt in neck.
Anxiety becoming worse.
Dizziness during attacks.
Vomited on one occasion.
Mother reports Pt becoming afraid to leave house in case attack occurs.
No identifiable triggers.
Dark room, ice – no effect.

Treatment:

31.01.18

Eletriptan 40 mg – orally on attack.
Ibuprofen 400 mg – 6 hrly (max t.d.s.).

Pt complained of drowsiness and diarrhoea since commenced on eletriptan.
No improvement in symptoms. Anxiety worse – Pt describes as 'panic' accompanying symptoms.
Mother concerned daughter becoming depressed; withdrawn, housebound, losing interest in activities & boyfriend, oversleeping, comfort eating → weight gain.
B.P 120/80

Treatment:

24.02.18

Amitriptyline 25 mg – b.d.

Pt presented alone. Complains of numbness and tingling (paraesthesia) in fingers 4 & 5 left hand.
No improvement in symptoms.
Pt reported car accident Jun 2017. Not previously disclosed as wishes to keep from mother.
Sustained 'whiplash injury'.
No treatment sought at time.
Referral to neurologist, for investigation and management.

Writing Task:

Using the information in the case notes, write a letter of referral to Dr Robert Edwards, a neurologist. Outline Ms Day's relevant history and request further investigations and management. Address the letter to Dr Robert Edwards, Rushford Hospital, Long Gully Road, Littletown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST:
TIME ALLOWED:

MEDICINE
READING TIME:
WRITING TIME:

5 MINUTES
40 MINUTES

Notes:

Patient: Erika Stone (Miss)
DOB: 06/04/1980
Residence: 3 Tollington Park, London, N4 3BD
Social Background: Single mother (10 y/o son)
Family History: Father – Diabetes type 2
Mother – Diabetes type 1
Medical History: 1999 – Diabetes type 2
Current Drugs: Metformin 500mg 2/24
Glipizide 5mg 2 mane
05/02/2018 Concerned that glu levels are not controlled – checks often (worried?)
Attends health centre – feels not taking her concerns serious
Recent blood sugar levels (BSL) 6 -18
Checks BP at home
Last eye check in October 2012 – OK
Full physical exam: NAD
O/E Medication added: candesartan (Atacand) tab 4mg 1 mane
Review 2/52
16/02/2018 Pathology report received:
FBE, U&E, creatinine, LFTs in normal range
GFR>60ml/min
HbA1c 10% (very poor control)
Lipids: Chol 6.2 (high), Trig 2.4, LDLC 3.7
29/02/2018 Review of pathology results w/ pt
Change in meds recommended
Metformin regime changed from 2 nocte to 1 b.d.
Atorvastatin (Lipitor) 10mg 1 mane added

Glipizide 5mg 2 mane added
Review 2/52
20/03/2018 Home BP in range (155/100)
Sugars ++
Pathology report received: Chol 3.2, trig 1.7, LDLC 1.1
30/03/2018 Pathology report r/v
Fasting sugar usually in 16+ (high) range
Other blood sugar 7-8
Refer to specialist at Diabetes Unit for further management of
sugar levels

Writing task:

Using the information in the case notes, write a letter of referral to Dr Maria Foreman, an endocrinologist at the Endocrinology department of Royal Restricted Hospital, 11 Lake Street, London, NW1 2TG.

In your answer:

- Expand the relevant notes into complete sentences.
- Do not use note form.
- Use letter format.

The body of the letter should be approximately 180-200 words.

Occupational English Test

WRITING SUB-TEST: MEDICINE

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

Mrs Priya Sharma is a patient in your general practice who is concerned about her glucose level control.

PATIENT DETAILS:

Name: Mrs Priya Sharma
DOB: 08.05.58 (Age 60)
Address: 71 Seaside Street, Newtown

Social background:

Married 40 years – 3 adult children, 5 grandchildren (overseas).
Retired (clerical worker).

Family history: Many relatives with type 2 diabetes
Nil else significant

Medical history: 1999 – type 2 diabetes
Nil significant, no operations
Allergic to penicillin Menopause 12 yrs
Never smoked, nil alcohol
No formal exercise

Current Drugs: Metformin 500mg b.d.
Glipizide 5mg 2 mane
No other prescribed, OTC, or recreational

29/12/18

Discussion: Concerned that her glucose levels are not well enough controlled – checks levels often (worried?)
Attends health centre – feels not taking her concerns seriously Recent blood sugar levels (BSL) 6-18
Checks BP at home
Last eye check October 2017 – OK Wt steady, BMI 24
App good, good diet
Bowels normal, micturition normal

O/E: Full physical exam: NAD
BP 155/100
No peripheral neuropathy; pelvic exam not performed
Pathology requested: FBE, U&Es, creatinine, LFTs, full lipid profile, HbA1c Medication added:
candesartan (Atacand) tab 4mg 1 mane
Review 2 weeks

05/01/19 Pathology report received:
FBE, U&Es, creatinine, LFTs in normal range
GFR > 60ml/min
HbA1c 10% (very poor control)
Lipids: Chol 6.2 (high), Trig 2.4, LDLC 3.7

12/01/19 Review of pathology results with Pt Changes in medication recommended
Metformin regime changed from 500mg b.d. to 750mg b.d.
Atorvastatin (Lipitor) 20mg 1 mane added Glipizide 5mg 2 mane
Review 2 weeks

30/01/19 Home BP in range Sugars improved
Pathology requested: fasting lipids, full profile

06/02/19 Pathology report received: Chol 3.2, Trig 1.7, LDLC 1.1

10/02/19 Pathology report reviewed with Mrs Sharma
Fasting sugar usually in 16+ (high) range
Other blood sugars 7-8
Refer to specialist at Diabetes Unit for further management of sugar levels

Writing Task:

Using the information in the case notes, write a letter of referral to Dr Smith, an endocrinologist at City Hospital, for further management of Mrs Sharma's sugar levels. Address the letter to Dr Lisa Smith, Endocrinologist, City Hospital, Newtown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180–200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: MEDICINE

TIME ALLOWED: READING TIME : 5 MINUTES

WRITING TIME : 45 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

You are a plastic surgeon at Newtown Hospital. Doris White a 64 years woman.

Patient: Doris White

Address: 5 Woods St, Stillwater, Newtown city

D.O.B. 01.04.1955

Social history: lives alone with her cat

Independent use frame

On social service registry (for laundry and cleaning) Good mental health

Past medical history: type 2 diabetes

Chronic renal failure on regular haemodialysis Ischaemic heart disease ,
hypertension

Peripheral vascular disease , hyperpigmented skin area

Medications: Aspirin 75 mg once

Atorvastatin 40 mg once

Insulin 50 iu 2 times

Beta blockers 5mg 2 times

Linezolid 600mg

Nitroglycerin 4mg PND

Calcium 300 mg 2 times

Multivitamins 2 tablets

25/03/2019

Hospital admission, neglected traumatic leg injury (slip in bathroom)
Treated with analgesics (paracetamol), antibiotic (amoxicillin) and topical pressure therapy

10/04/2019

Unhealed wound , so surgical debridement done Ankle Brachial index = 60

17/04/2019

No improvement

Skin graft with flap under general anaesthesia. 25/04/2019

Graft healing 40% (with epithelium growth) Wound is hyperbaric oxygen.
ABI = 70

03/05/2019

Graft healing 50%

Negative pressure dressings used.

18/05/2019

Graft healing 70% ,so plan for discharge. ABI = 70

Plan : discharge

Attach list of meds

Writing task

Using the information in the case notes, write a letter to Mrs Lucy Walters , a Community Nurse , requesting for wound dressing. Address the letter to Mrs Lucy Walters, Newtown Nurse Clinic, 10 Stillwater St, Newtown.

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TIME :

READING: 5 MINUTES

WRITING : 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

You are a GP and Ms. Topp has been following with you for 20 years.

Patient name:. Katherine Topp

Age: 59

Social History: unmarried, no partner, lives with mother

Librarian

2011- left work to care for mother

Family history: Mother died ovarian cancer (August 2017)

Past medical history: Depression since early 20s (not on medication

Suffers from migraines, Reynaud's syndrome and IBS.

Takes OTC medications for them(Migraleve, ginkgo biloba, Buscopan)- not effect anymore

Menopause at 52- heavy bleeding one year before

2017- citalopram 20mg daily for depression- effective

Presenting complaints:

Generalized fatigue, joint pain, abdominal distress

15/6/2019:

Subjective: generalised weakness, bone pain, sleeping disturbance (due to pain), bloating & constipation

Morning stiffness, cold intolerance in feet and hands.

Weakness more on right side

Objective: joint tenderness, no joint swelling, loss of balance.

Diagnosis : ? fibromyalgia

Blood tests (2014, 2017, 2019) NAD Colonoscopy (2016) NAD

Plan: refer to Rheumatologist, attach lab results

Writing Task

Using the information in the case notes , write a letter of a referral to Dr
Gloves , a rheumatologist , Bayview Private Hospital , 81 Canyon Road ,
Bayview

Task: refer Ms. Topp to Dr....., A Rheumatologist at..... For diagnosis and
treatment

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WRITING SUB-TEST

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TIME :

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Read the case notes below and complete the writing task which follows.

Notes:

You are a physician at the Emergency Department.

Patient name: Mr Peter Sullivan

DOB: 15/08/1963

Age : 56

Social History: single – no children
lives with elderly parent

Mother and father main carer

Livestock farmer , works long hours in the sun

Past medical history: 2006- excision malignant melanoma

2011- excision compound naevus- confirm by histology examination - no action taken

2015- two ribs fracture

2016- forearm fracture

Medications: nil

Family history : moderate dementia – mother

07/07/2019

Presenting complaint:

He came to emergency department - severe right arm pain - by kick from horse in farm

Subjective : bruise , swelling

Ulnar fracture confirmed by x-ray

Medications : Paracetamol

Arm sling - splint

Objective: large mole on his right shoulder - 6mm x 7mm - irregular edges - variable colours - light brown to dark. Some irritation

Other Large raised mole on his neck - 5mm x 7mm - dark red - no oozing or bleeding.

Plan : urgent referral to a dermatologist – biopsy.

Review after 2 weeks for his ulnar fracture

Writing Task

Using the information in the case notes , write a letter of a referral to Dr Patrick Silvester . Address the letter to Dr Patrick Silvester , Consultant Dermatologist ,Newtown Hospital , 81 Canyon Road , Newtown.

In your answer:

- **Expand the relevant notes into complete sentences.**
- **Do ~~not~~ use note form.**
- **Use the letter form.**

The body of the letter should be approximately 180 – 200 words.

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Read the case notes below and complete the writing task which follows.

Notes:

You are a cardiothoracic surgeon.

Patient name: Ms Janet Pristiely

DOB: 19/07/1947

Age : 72

Social History:

widow - retired accountant care of 5 grand children

non alcoholic – smokes 35 cigarrates/d

Past medical history:

Hypertension – DM II

Depression , Nephropathy Ischemic heart disease

Acute myocardial infarction (2018) **Medications**

Aspirin 75 mg

Beta blockers 10 mg

Citalopram 10 gm

Insulin 50 IU

Statins 40 mg

02/08/2019

Non-critical coronary artery bypass cancelled

Procedure not done due to smoking prior admission

On admission BP was 180/100 , ↑ Betablockers

BP fall to 140/80

Psychiatrist Counselling:

smoking cessation.

social and financial status including grand children

diagnosed as moderate depression

03/08/2019

Discharge Plan:

Lisinopril 8 mg , monitor BP& electrolyte 1w

Nicotine patches 25 mg

↑ Citalopram 20 mg

↑ Aspirin 150 mg , stop on re-admission

Re-admission advice on 19/10 :

Respite care for grand children

Contact me if SOB , chest pain

Writing Task

Using the information in the case notes , write a letter of a discharge to the patient's GP including history ,medication and advice . Address the letter to Dr Peter Harvey ,Newtown Clinic , 62 Ville Road , Newtown.

In your answer:

- **Expand the relevant notes into complete sentences.**
- **Do not use note form.**
- **Use the letter form.**

The body of the letter should be approximately 180 – 200 words.

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TIME :

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Read the case notes below and complete the writing task which follows.

Notes:

Patient name: Mr Jane Browne

DOB: 25/08/2001

Age : 18

History:

Student

Facial eczema as baby

Flexular eczema as child

Allergic rhinitis to pollens , cat hair

History of asthma (mother)

01/09/2019

Tender ,weepy, crusty nipple

Surrounding inflammation from plaster dressing

Treatment plan:

Hydrocortisone

Soap substitute

Ellomite e.g Edomin 5

stop using plaster , replace it with mepor dressing and keep uncovered except during sport

On 10/9/2019

Showed improvement

Feels happy with the mepor

Advised to return in case of symptoms worsening

On 21/9/2019

Nipple never healed

Increased friction with T-shirts during sport , so cover with mepor

Feels embarrassed after sports dressings room , need complete resolution

Reported use hydrocortisone intermittently

On examination

Nipple was tender , no crustations , change in appearance and become warty

Diagnosis: ? Seborrheic keratosis

: ? Eczema

: ? Others

Plan : refer to a dermatologist for assessment and diagnosis

Writing Task

Using the information in the case notes , write a letter of a referral to Dr Elizabeth Macnor ,Consultant Dermatologist ,Newtown Hospital , 81 Canyon Road , Newtown.

November 2018 (Writing Case note)

Allen Mathis

61 yo retired cabinet maker.

Personal History:

Married, lives with family

Ex-smoker (smoked for 35 yrs, quitted 7yrs ago)

Mother and grandmother had DM.

Medical history:

Poorly controlled DM

Mild-Moderate HTN

Ischemic Heart disease

2011: had coronary artery angioplasty after acute MI

2012: cataract surgery

Fungal infection on foot. Went po— therapy

FBG - > 5-25mmol

HbA1c -10%

Presenting complaints:

Worsening of Dyspnea, decreased exercise tolerance, ankle edema for 2 months.

Can't sleep lie down (orthopnea) for past few days.

No chest pain .

Medications:

Inj. Novorapid

Inj. Lantus

Metformin

Metoprolol

Aspirin

Rosuvastatin

Nifedipine

Hydrochlorothiazide

3.11.18

Objectives:

Temp : 36.8

BMI : obese>30

BP 148/98

Chest : apex beat shifted laterally with ejection systolic murmur. Bibasal crepitations .

Bilateral Ankle edema

ECG: T inversion in antero-lateral leads. No Q wave, no ST elevation (changed from 6 months ago)

Assessment : Biventricular heart failure with possible silent ischemia with sustained HTN and poorly controlled DM

Plan: refer to emergency cardiologist for urgent assessment and advice on management of acute heart failure and possible silent myocardial Ischemia.

Referral letter to

Cardiologist,

Emergency department

Central hospital

Stillwater.

For urgent assessment and advice on Mx.